07-25 Dual-Track Business Model for Community Revitalization

Strategic Brief

Community Revitalization via Dual-Track Business Models

A symbiotic ecosystem pairing AI-driven small business consultancy with independent primary care clinics.

Small Business AI Consultancy

Rapid Prototyping Engine

Young teams use AI (e.g., Codex) to transform business interviews into functional prototypes in **15 minutes**.

The Distribution Strategy

Initial low-cost projects build a **relational layer** and trust network, paving the way for scalable enterprise solutions.

AI-Driven Sales · Youth Skill Development · Community Trust

Neepic

The Legacy EHR Replacement

"A voice-driven clinical platform allowing practitioners to focus entirely on patient care, not manual data entry."

New Primary Care Paradigm

Independent NPs

- **Clinician Front, College Back**
  Physical community clinics serving as dual assets for healthcare delivery and localized training.
- **Financial Abstraction**
  A "hedge fund" layer to pay practitioners immediately, bypassing the complex revenue cycle delays.
- **Pilot: Nancy's Practice**
  Initial launch in Women's Health/Midwifery to define the scope and prevent patient leakage.

Technical Infrastructure

Edge Computing

SOLUTION: THE UNIVERSAL BACKEND

Preventing "spreadsheet nightmares" via a standardized data model (inspired by Open EHR) that integrates custom front-ends without creating silos.

Patient Journey Stages

1. Acquisition
2. Guidance
3. Delivery
4. Feedback

Strategic Roadmap & Action Items

@THE TEAM

- ○ Define internal tool set & summary of camp projects
- ○ Prototype connections with Restoration Medicine simulation
- ○ Test clinical workflow acting as "Nancy's" patients
- ○ Evaluate Open EHR suitability for backend architecture

ADMIN & LEGAL

Investigate pathways for NP credentialing with **Medicaid, Medicare, and PPO** networks.

@KEITH

K Implementation of 'dial monkey' (IVR/telephony) system

The discussion outlines a dual-track business model aimed at community revitalization, centered around two core ventures: a software consultancy empowering young people to build AI-driven tools for small businesses, and a new primary care model for independent nurse practitioners. Both initiatives are designed to operate within a shared physical space, creating a symbiotic ecosystem that addresses local economic and healthcare needs while developing novel, scalable solutions.

The Small Business Software Consultancy Model

This section details a proposed consultancy where young people are trained to build custom, AI-powered software solutions for local small businesses. The model, successfully piloted at a recent “camp,” involves interviewing business owners, drafting product requirements, and using AI tools like Codex to generate functional prototypes in as little as 15 minutes. This approach was met with profound enthusiasm from participants like Amy, a real estate professional, who felt listened to and engaged in the creation of tools for her business. The strategy is to leverage these initial, low-cost projects to build a powerful relational layer and a “distribution network.” Rather than focusing on immediate profit, the goal is to establish trust and prove value, which can later be scaled into larger, more profitable enterprise solutions. This initiative also serves as a new career pathway for youth, equipping them with modern sales, product, and AI skills in a hands-on environment that bridges community divides by getting people to build tangible solutions together.

The Data Silo Problem and Backend Standardization

This section addresses a critical technical challenge arising from the consultancy model: the risk of creating a “spreadsheet nightmare” where numerous disconnected, custom-built applications lead to data silos and integration failures. The speakers acknowledge that while AI-generated apps are more powerful than spreadsheets, they can reproduce the same fragmentation problem. The consensus is that the solution lies not in forcing a uniform user interface but in developing a standardized, universal backend architecture with a consistent data model. This approach, inspired by platforms like Palantir and open-source frameworks like Open EHR, would allow for the rapid creation of custom front-ends while ensuring all data remains integrated and scalable. The team plans to achieve this by iterating through app builds with several businesses in the same vertical (e.g., real estate) to identify common patterns and define a robust, “AI-ready” backend. This distributed architecture would prevent silos and form the technical foundation for a scalable, interoperable ecosystem.

Vision for a New Primary Care Model

This section outlines a comprehensive vision for reinventing primary care by empowering independent nurse practitioners. The model is built around establishing physical community clinics—with a “clinician in the front, college in the back”—that serve as vital community assets. A key innovation is a financial abstraction layer, or “hedge fund,” designed to pay practitioners immediately for their services, while a central company handles the complex backend of billing and revenue cycle management. The long-term financial goal is to move away from fee-for-service toward bundled payments and eventually underwrite the health risks of entire populations. At the core of this vision is the development of “Neepic,” a nextgeneration, voice-driven clinical software platform intended to replace obsolete EHRs like Epic. This system aims to create a superior working experience for practitioners by eliminating manual data entry and allowing them to focus entirely on patient care. The overall approach is a paradigm shift, intended to build a new, viable model from the ground up rather than making incremental changes to a broken system.

The Clinical Workflow and System Architecture

This section delves into the practical architecture and workflow for the proposed primary care model, breaking down the patient journey into four key stages: 1) Patient Acquisition and Onboarding, 2) Care Guidance and Planning, 3) Care Delivery, and 4) a closed Feedback Loop. The system's design emphasizes the integration of patient-provided information, biometric data from wearables, and voice interactions, which legacy EHRs fail to handle. A critical design principle is to abstract away billing and coding from clinicians; the voice-driven software will capture clinical details, and AI agents will generate the necessary codes for reimbursement, ensuring the focus remains on clinical accuracy. The proposed architecture is distributed and based on edge computing, where patient data remains localized on personal devices rather than being stored in a centralized, monolithic EMR. This approach, which challenges the "HIPAA world" of complex data-sharing agreements, is seen as more secure and efficient, avoiding the systemic failures and data silos that plague current healthcare technology.

Go-to-Market Strategy and Ecosystem Integration

This section focuses on the strategy for launching and scaling the healthcare venture by creating a holistic, integrated ecosystem. The plan will start with a pilot practice led by "Nancy," a midwife, focusing on the specific domain of women's health to create a defined and manageable initial scope. This central hub will feature in-office testing and handheld imaging devices to prevent patient leakage and will include a "call booth" for comfortable, on-site virtual consultations with specialists, ensuring the primary practitioner remains the "quarterback" of care. The strategy involves rewiring the healthcare delivery system by partnering with virtual specialty networks (e.g., Restoration Medicine), innovative service providers like Mark Cuban's pharmacy, and eventually self-insured employers. This will create a high-value, direct-care network that bypasses the inefficiencies of the traditional system. The model will also generate additional revenue streams through a direct-to-consumer brand offering related courses, educational content, and products.

Action Items

@The Team

- [ ] Define the internal tool set and create a summary/debrief of the camp projects - [TBD]
- [ ] Prototype the three key connections (practitioner role, patient-focused app, and network connectivity) using a simulation with Restoration Medicine - [TBD]
- [ ] Execute a prototype simulation by acting as Nancy’s patients to test the clinical workflow - [TBD]
- [ ] Investigate and define the specific legal and administrative pathways for credentialing an independent nurse practitioner with Medicaid, Medicare, and PPO networks - [TBD]
- [ ] Research and evaluate the suitability of Open EHR as the backend architecture for the new system - [TBD]
- [ ] Define the specific modules and general architecture for the new “Neepic” backend system to replace legacy EHR infrastructure - [TBD]
- [ ] Conduct research to define the specific credentialing and billing requirements for independent nurse practitioners to operate outside of large hospital systems - [TBD]

@Keith

- [ ] Handle the implementation/configuration of the ‘dial monkey’ (IVR/telephony) system - [TBD]